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SURGICAL PATHOLOGY

Order:

Component
Surgical Pathology
(AP)

2 mo ago
COLUMBIA UNIVERSITY PATHOLOGISTS
630 West 168th Street New York, New York
Fax#

Name:

D.O.B.
Sex: F Physician:
Location: MAN 51W51 300 ULTRAS 51 W 51st Street, #300
Columbia Doctors Midtown Radiology
New York, NY

SURGICAL PATHOLOGY OUTREACH
See Addendum/Procedure
SPECIMEN: Breast, right at 2:00 2cm fn, USG core biopsy

CLINICAL INFORMATION:
Order diagnosis: Encounter for follow-up examination after
completed treatment for conditions other than malignant neoplasm.
Right breast, 2:00, 2cm FN irregular mass on sono ? granulomatous
mastitis, stromal fibrosis r/o ILC/IDC.

GROSS DESCRIPTION:
The specimen is received fixed in formalin in a container labeled
with the patient's name, irregular mass on sono ? granulomatous mastitis stromal fibrosis
r/o ILDC/IDC". It consists of 3 yellow, white, and focally
hemorrhagic breast tissue cores ranging from 0.5-2.3 cm in greatest
dimension. The specimen is submitted in toto in one cassette
labeled A1.

Cold ischemic time: Less than 1 minute
Time in formalin: 1:35 PM on 10.14.2025
Total time in formalin: 26 hours 25 minute

adm adm

DIAGNOSIS (ES) :
Breast, right at 2:00 2 cm fn, USG core biopsy:
Ductal carcinoma in situ (DCIS), nuclear grade 3 and 2-3,
cribriform pattern, with comedo necrosis and focal calcifications,
involving multiple ducts, of female mammary gland, p63 and SMMS-1
highlight primarily the presence of or attenuated presence of
myoepithelial cells around the DCIS. The pan-CK immunostain

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highlight carcinoma and non neoplastic epithelial cells. While definite evidence of invasion is not seen evaluation is hampered/limited by extensive cancerization of lobules by DCIS, myoepithelial cells attenuation and stromal inflammation. Background shows foci of lymphocytic lobulitis. ER and PR immunostains pending.

Date Dictated: 10.16.25

vg
This report has been electronically signed and results reported on 15:20 by Hanina Hibshoosh, MD .

Reviewed by: Attending:

The diagnosis was rendered by the attending

Receptor Expression Analysis Date Ordered:
Status: Signed Out

INTERPRETATION
Test Performed on formalin fixed paraffin embedded section of:
Right breast, 2:00, 2 cm FN, core biopsy

The results are for in situ carcinoma.

Specimen part: A

Slide#: A1

Results:
ADEQUATE FIXATION:

FIXATION:
 Ischemic time: <1 min
ADEQUATE FIXATION:

FIXATION:
 Duration of fixation (adequate 6-72 hours): 26 hours and 25 mm
Fixative: 10% neutral buffered formalin

STAINING CONTROLS:
 ER external control positive: Yes
STAINING CONTROLS:
 ER internal control positive

NEGATIVE for estrogen receptor expression (suggest repeat testing

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on excision)

No [redacted] an immunohistochemical stain utilizing an anti-estrogen receptor antibody (Ventana clone SP1). The specimen was adequate for evaluation for ER as per ASCO/CAP guidelines 2020 (see reference below). (Note: This assay has not been validated on decalcified tissues. If performed on a decalcified specimen, negative results should be interpreted with caution).

Reference:
Arch Pathol Lab Med. 2010 Jul;134(7):e48-72. American Society of Clinical Oncology/College of American Pathologists guideline recommendations for immunohistochemical testing of estrogen and progesterone receptors in breast cancer (unabridged version). Hammond ME, Hayes DF, et. al.

STAINING CONTROLS:
PgR external control positive: Yes
STAINING CONTROLS:
PgR internal control positive

NEGATIVE for progesterone receptor expression.

No [redacted] an immunohistochemical stain utilizing an anti-progesterone receptor antibody (Leica clone 16). The specimen was adequate for evaluation for PR as per ASCO/CAP guidelines 2020 (see reference below). (Note: This assay has not been validated on decalcified tissues. If performed on a decalcified specimen, negative results should be interpreted with caution).

Reference:
Arch Pathol Lab Med. 2010 Jul;134(7):e48-72. American Society of Clinical Oncology/College of American Pathologists guideline recommendations for immunohistochemical testing of estrogen and progesterone receptors in breast cancer (unabridged version). Hammond ME, Hayes DF, et. al.

[redacted]

This procedure/addenda has been electronically signed and results reported on [redacted] 10:2 by Hanina Hibshoosh, MD.

2 mo ago

[REDACTED] [REDACTED] [REDACTED]

ADDENDUM

This case has been reviewed in Memorial Sloan Kettering Cancer Center (their case # S25-092449) and their diagnosis of the right breast biopsy is that there is 1 focus of microinvasive carcinoma, as well as ductal carcinoma in situ, intermediate to high nuclear grade, with cribriform pattern with necrosis and calcifications.

Note: please see our original report citing ductal carcinoma in situ. Regarding microinvasion: complete exclusion of microinvasion is not possible but nor is it definitive in our opinion.

████████████████████

This procedure/addenda has been electronically signed and results reported on [REDACTED] 15:2 by Hanina Hibshoosh, MD.

_____, _____, _____

Resulting Agency	NYP_Columbia
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Specimen Collected: [REDACTED] 13:35

Performed by: NYP HOSPITAL COLUMBIA LAB

Received From: Weill Cornell Medicine, Columbia

Physicians, and New York Presbyterian Hospital

Last Resulted: [REDACTED] 15:20

Result Received: [REDACTED] 07:06

View Encounter Received Information

Order Tracking

